

Medical Billing Course Syllabus

Module 1: Introduction to Medical Billing

1. Overview of Healthcare System
 2. Role of a Medical Biller
 3. Difference between Medical Billing & Medical Coding
 4. Healthcare Payers: Government vs. Private Insurance
 5. HIPAA and Patient Confidentiality
-

Module 2: Healthcare Fundamentals

1. Basic Medical Terminology
 2. Common Anatomy & Physiology Terms for Billers
 3. ICD, CPT, and HCPCS Codes Overview
 4. Medical Documentation & Health Records (EHR/EMR)
 5. Medical Necessity & Compliance
-

Module 3: Insurance Basics

1. Types of Health Insurance Plans (HMO, PPO, Medicare, Medicaid, Workers' Compensation)
 2. Insurance Terminology (Premiums, Deductibles, Copays, Coinsurance)
 3. Eligibility Verification & Insurance Coverage
 4. Coordination of Benefits (COB)
 5. Prior Authorization & Referrals
-

Module 4: The Medical Billing Cycle

1. Patient Registration & Data Entry
 2. Insurance Verification Process
 3. Charge Capture & Coding Basics
 4. Preparing and Submitting Claims (Paper vs. Electronic)
 5. Payment Posting & EOB (Explanation of Benefits)
 6. Claim Follow-up and Denial Management
 7. Collections & Patient Billing
-

Module 5: Claims Processing

1. CMS-1500 Form Completion
2. UB-04 (Institutional Billing) Basics

3. Clearinghouses & Electronic Claims Submission (EDI)
 4. Claim Scrubbing & Error Prevention
 5. Timely Filing Limits
-

Module 6: Reimbursement Methodologies

1. Fee-for-Service vs. Value-Based Reimbursement
 2. DRGs (Diagnosis Related Groups)
 3. APCs (Ambulatory Payment Classifications)
 4. Capitation & Global Billing
 5. Medicare & Medicaid Billing Guidelines
-

Module 7: Advanced Billing & Compliance

1. Fraud, Waste, and Abuse in Medical Billing
 2. OIG (Office of Inspector General) Compliance Guidelines
 3. Appeals and Grievance Process
 4. Corrective Action Plans for Errors
 5. Audits and Documentation Requirements
-

Module 8: Technology in Medical Billing

1. Introduction to Practice Management Software
 2. Electronic Health Records (EHR) Billing Integration
 3. Common Billing Software (Kareo, AdvancedMD, eClinicalWorks, etc.)
 4. Telehealth Billing Guidelines
 5. Emerging Trends in Medical Billing (AI, Automation, RCM)
-

Module 9: Specialty Billing

1. Hospital Billing vs. Physician Billing
 2. Laboratory & Radiology Billing
 3. Mental Health & Behavioral Health Billing
 4. Dental Billing Basics
 5. Durable Medical Equipment (DME) Billing
-

Module 10: Career Preparation

1. Job Roles in Medical Billing (Biller, AR Specialist, RCM Executive)
2. Resume & Interview Preparation
3. Freelance Medical Billing & Outsourcing Opportunities

4. Certification Exams (CMB, CMRS, AAPC Billing Certification)
5. Continuing Education & Professional Growth

Overview of Healthcare System (Medical Billing Context)

Medical billing exists **within the larger healthcare system**, so understanding how the system works is the first step.

1. What is a Healthcare System?

A healthcare system is the organized structure through which **medical services are delivered, financed, and regulated**. It includes:

- **Providers** → doctors, hospitals, clinics, laboratories, pharmacies.
 - **Payers** → insurance companies, government programs (Medicare, Medicaid), self-pay patients.
 - **Patients** → the consumers of healthcare services.
 - **Regulatory bodies** → organizations like CMS (Centers for Medicare & Medicaid Services), WHO, and local health authorities.
-

2. Key Components of Healthcare System

1. **Healthcare Providers** – Physicians, nurses, hospitals, clinics, labs, imaging centers.
 2. **Healthcare Payers** –
 - Government: Medicare, Medicaid, VA, Workers' Compensation.
 - Private Insurance: Blue Cross, Aetna, UnitedHealth, etc.
 - Self-pay patients (no insurance).
 3. **Healthcare Services** – Primary care, specialty care, inpatient, outpatient, emergency, preventive services.
 4. **Healthcare Financing** – How medical services are paid for (insurance premiums, government funds, employer coverage).
 5. **Healthcare Regulation** – Laws and compliance (HIPAA, Affordable Care Act, OSHA).
-

3. U.S. Healthcare System (as an example)

- **Multi-payer system:** Government + private insurance.
 - **Key government programs:**
 - **Medicare** → For people 65+ and certain disabilities.
 - **Medicaid** → For low-income individuals/families.
 - **CHIP** → Children's Health Insurance Program.
 - **Private insurance:** Offered through employers or purchased individually.
-

4. Global Healthcare Models (for awareness)

Different countries use different healthcare systems:

- **Beveridge Model** (UK, Spain): Government provides healthcare, funded by taxes.
 - **Bismarck Model** (Germany, Japan): Insurance-based, funded by employers & employees.
 - **National Health Insurance Model** (Canada): Government provides insurance, private providers deliver care.
 - **Out-of-Pocket Model** (many developing countries): Patients pay directly.
-

5. Connection to Medical Billing

- Medical billing ensures **providers get paid** by payers for services delivered.
 - Every healthcare system has its own **rules, codes, and reimbursement structures**.
 - Billers must understand **which payer is responsible, what documents are needed, and how claims are submitted**.
-

☒ In summary:

The healthcare system is a network of providers, patients, payers, and regulators. Medical billing acts as the **bridge between patient care and financial reimbursement**, making knowledge of the healthcare system essential for a biller.

1. Role of a Medical Biller

Medical billers act as the **financial bridge** between healthcare providers, patients, and insurance companies.

Key Responsibilities:

- **Patient Data Entry** → Collect & enter demographics, insurance info, medical history.
- **Verify Insurance** → Check eligibility & benefits before treatment.
- **Prepare Claims** → Translate coded medical procedures (from coders) into billing claims.
- **Submit Claims** → Send to insurance (electronically or paper).
- **Track Claims** → Monitor status, follow up on unpaid or rejected claims.
- **Post Payments** → Record insurance payments, patient copays, and adjustments.
- **Handle Denials/Appeals** → Correct errors & resubmit denied claims.
- **Patient Billing** → Prepare statements, answer billing inquiries, and arrange payment plans.

☐ **In short:** A medical biller ensures that **healthcare providers get paid accurately and timely**.

2. Difference Between Medical Billing & Medical Coding

Although related, billing and coding are **two distinct professions**:

Aspect	Medical Coding	Medical Billing
Definition	Translating medical services, diagnoses, and procedures into standardized codes (ICD, CPT, HCPCS).	Using those codes to create, submit, and follow up on insurance claims and patient bills.
Role	Focuses on the clinical side – accuracy of coding based on physician notes and medical records.	Focuses on the financial side – ensuring providers are reimbursed properly.
Tools Used	ICD-10-CM, CPT, HCPCS manuals/software.	Practice Management Systems, Billing Software, Clearinghouses.
Goal	Ensure accurate representation of patient's care.	Ensure provider receives correct reimbursement.
Example	Appendectomy surgery = CPT code 44950, ICD-10 code K35.80.	Submitting a claim with those codes to insurance for payment.

□ Think of it like this:

- **Coder** = Translates medical info → codes.
 - **Biller** = Translates codes → money.
-

3. Healthcare Payers: Government vs. Private Insurance

Medical billing always involves **payers** who reimburse providers. These are mainly **government** or **private**.

A. Government Payers (Public Insurance)

- Funded by **taxes**.
 - Coverage is regulated by law.
 - Examples (U.S.):
 - **Medicare** → 65+ and disabled patients.
 - **Medicaid** → Low-income individuals/families.
 - **CHIP** → Children's health insurance program.
 - **Tricare / VA** → Military & veterans.
 - **Pros**: Standardized rules, large coverage base.
 - **Cons**: Lower reimbursement rates, strict regulations.
-

B. Private Payers (Commercial Insurance)

- Funded by **employers or individuals**.
- More flexible, but policies differ.
- Examples: Blue Cross Blue Shield, Aetna, Cigna, UnitedHealthcare.
- **Pros:** Higher reimbursement, more plan options.
- **Cons:** Complex rules, varied coverage, frequent claim denials.

Key Differences at a Glance

Feature	Government Insurance	Private Insurance
Funding Source	Taxes (federal/state)	Premiums paid by employers/individuals
Eligibility	Based on age, income, disability	Based on policy purchased
Coverage	Standardized benefits	Varies by plan & company
Reimbursement	Lower, fixed fee schedules	Higher, negotiable rates
Regulation	Strict federal/state rules	Company policies + state laws

✓ Summary:

- **Medical Biller** = ensures smooth reimbursement cycle.
- **Billing vs Coding** = clinical coding vs financial reimbursement.
- **Payers** = government (tax-funded, standardized) vs private (premium-funded, varied).

HIPAA and Patient Confidentiality

1. What is HIPAA?

HIPAA stands for **Health Insurance Portability and Accountability Act of 1996** (U.S. law).

- Created to **protect patient health information**.
- Ensures data is **confidential, accurate, and only shared when necessary**.
- Applies to all healthcare professionals, including **medical billers and coders**.

2. Protected Health Information (PHI)

PHI = Any **personally identifiable health information**.

Examples include:

- Name, address, phone number
- Social Security number
- Medical record number
- Diagnoses, treatments, medications
- Insurance & billing details

- If information can identify the patient + relates to health → it's **PHI**.
-

3. HIPAA Privacy Rule

- Controls **who can access patient data**.
 - Patients have rights to:
 - View their medical records.
 - Request corrections.
 - Decide who can access their data.
 - Medical billers can **only use PHI for billing and reimbursement purposes**.
-

4. HIPAA Security Rule

- Focuses on **electronic health information (ePHI)**.
 - Requires:
 - Strong passwords & user authentication.
 - Data encryption.
 - Secure email or EHR systems.
 - Firewalls & antivirus protection.
-

5. HIPAA in Medical Billing

Medical billers handle **sensitive data** daily (insurance IDs, diagnoses, treatments).

- Must **only share with authorized persons** (insurance companies, providers).
 - Must **avoid discussing patient info in public places**.
 - Must **store records securely** (locked cabinets or encrypted systems).
 - Must **shred or securely delete old records**.
-

6. Patient Confidentiality

- The **ethical and legal obligation** to keep patient info private.
 - Even outside the workplace, billers **cannot disclose patient information**.
 - Confidentiality applies to:
 - Paper records
 - Electronic files
 - Phone conversations
 - Emails
-

7. HIPAA Violations & Penalties

If a biller or provider breaks HIPAA rules:

- **Civil penalties** → fines up to **\$50,000 per violation**.
- **Criminal penalties** → fines up to **\$250,000** + possible jail time.
- Loss of job or certification.

Examples of Violations:

- ✗ Talking about a patient's treatment in public.
 - ✗ Leaving billing files unattended.
 - ✗ Sending patient info through unencrypted email.
-

☑ **Summary:**

- HIPAA ensures **patient information stays private & secure**.
- Medical billers must **use PHI only for billing**.
- Violations can lead to **serious fines, job loss, and legal action**.

1. Basic Medical Terminology (for Medical Billing)

Medical terminology is like the **language of healthcare**. Billers don't need to memorize everything doctors know, but they must understand common **prefixes, suffixes, and root words**.

a. Prefixes (Beginning of word)

- **Cardio-** = heart (e.g., cardiology)
- **Neuro-** = nerves (e.g., neurology)
- **Hemo-** = blood (e.g., hemoglobin)
- **Gastro-** = stomach (e.g., gastritis)

b. Suffixes (End of word)

- **-itis** = inflammation (e.g., arthritis = inflammation of joints)
- **-ectomy** = surgical removal (e.g., appendectomy = removal of appendix)
- **-logy** = study of (e.g., biology)
- **-pathy** = disease (e.g., neuropathy = nerve disease)

c. Root Words

- **Derm** = skin
- **Osteo** = bone
- **Pulmo** = lung
- **Hepat** = liver

☐ **Why important?** Billers must understand these to correctly interpret **physician notes, diagnoses, and procedures**.

2. Common Anatomy & Physiology Terms for Billers

Billers don't diagnose, but they must know **basic body systems** because insurance claims depend on them.

Major Body Systems (with examples):

1. **Cardiovascular System** – Heart, blood, vessels (e.g., hypertension, angioplasty).
2. **Respiratory System** – Lungs, trachea, bronchi (e.g., asthma, pneumonia).
3. **Digestive System** – Stomach, intestines, liver (e.g., gastritis, colonoscopy).
4. **Musculoskeletal System** – Bones, muscles, joints (e.g., fracture, arthritis).
5. **Nervous System** – Brain, spinal cord, nerves (e.g., epilepsy, neuropathy).
6. **Urinary System** – Kidneys, bladder (e.g., kidney stones, UTI).
7. **Reproductive System** – Male & female organs (e.g., hysterectomy, prostate cancer).
8. **Endocrine System** – Hormones, glands (e.g., diabetes, thyroid disorders).
9. **Integumentary System** – Skin, hair, nails (e.g., dermatitis, melanoma).

☐ **Why important?** If a claim shows a code for “appendectomy,” a biller should know it's surgery on the digestive system.

3. ICD, CPT, and HCPCS Codes Overview

Medical billing is **code-driven**. Codes translate medical services into **standardized numbers** for insurance claims.

a. ICD (International Classification of Diseases)

- Developed by **WHO**.
- Currently using **ICD-10-CM** in the U.S.
- Used for **diagnosis codes** (why the patient was treated).
- Example:
 - **E11.9** = Type 2 diabetes without complications
 - **J45.909** = Unspecified asthma

☐ **ICD** = “What's wrong with the patient?”

b. CPT (Current Procedural Terminology)

- Developed by the **American Medical Association (AMA)**.
- 5-digit codes for **procedures & services**.
- Used for physician, outpatient, and surgical services.
- Example:
 - **99213** = Office visit, established patient
 - **93000** = Electrocardiogram (ECG)

☐ CPT = “What service/treatment did the doctor perform?”

c. HCPCS (Healthcare Common Procedure Coding System)

- Based on CPT, but with **extra codes** (mainly for Medicare/Medicaid).
- Includes **supplies, equipment, drugs, ambulance services**.
- Example:
 - **A0429** = Ambulance service, basic life support
 - **J1050** = Injection, contraceptive

☐ HCPCS = “What supplies/equipment were used?”

Key Difference at a Glance

Code Set	Purpose	Example
ICD-10-CM	Diagnosis (disease/condition)	E11.9 = Type 2 Diabetes
CPT	Procedures/medical services	99213 = Office Visit
HCPCS	Supplies/equipment, Medicare billing	A0429 = Ambulance Service

☒ Summary for Billers:

- **ICD** = Diagnosis (why)
- **CPT** = Procedures (what service)
- **HCPCS** = Supplies/equipment (extras)

1. Medical Documentation & Health Records (EHR/EMR)

A. What is Medical Documentation?

- Medical documentation is the **official record** of a patient’s medical history, diagnoses, treatments, and outcomes.
- It serves as the **legal proof** of care and the **basis for billing**.

Purposes of Medical Documentation:

1. **Patient Care** → Helps providers track history and make informed decisions.
 2. **Legal Record** → Protects providers in case of malpractice claims.
 3. **Billing & Reimbursement** → Insurance requires documentation to prove services were medically necessary.
 4. **Research & Education** → Data used for medical studies, public health.
-

B. Types of Health Records

1. **Paper Records** → Traditional handwritten files (still used in some clinics).
2. **Electronic Medical Records (EMR)**
 - Digital version of patient chart in **one practice**.
 - Used internally by one doctor/clinic.
 - Example: A family doctor's EMR has notes, test results, and prescriptions.
3. **Electronic Health Records (EHR)**
 - A **broader digital record** that can be shared **across multiple providers**.
 - Contains labs, imaging, referrals, hospital visits.
 - Example: If a patient goes from family doctor → hospital → pharmacy, the EHR follows them.

□ Difference in simple terms:

- **EMR** = One doctor's record (limited).
 - **EHR** = Shared record across providers (comprehensive).
-

C. EHR/EMR in Medical Billing

- Billers use **EHR/EMR systems** to:
 - Access provider notes & coding information.
 - Generate claims directly from patient encounters.
 - Verify insurance info.
 - Reduce errors by linking documentation with billing software.
 - Common systems: Epic, Cerner, Kareo, eClinicalWorks, Athenahealth.
-

2. Medical Necessity & Compliance

A. What is Medical Necessity?

- A service is considered “**medically necessary**” if it is:
 1. **Reasonable & necessary** for diagnosis or treatment.
 2. Matches the patient's medical condition.
 3. Not performed primarily for convenience (of provider or patient).
 4. Consistent with accepted medical standards.

Example:

- ☒ Ordering an X-ray for a broken leg = Medically necessary.
- ☐ Ordering an X-ray for a mild headache = Not medically necessary.

☐ Insurance companies only reimburse for **medically necessary services**.

B. Compliance in Medical Billing

Compliance = Following the rules, laws, and guidelines in billing.

Key Compliance Areas:

1. **HIPAA** → Protect patient privacy.
 2. **False Claims Act** → Don't bill for services not provided.
 3. **Coding Accuracy** → Use correct ICD, CPT, HCPCS codes.
 4. **OIG (Office of Inspector General) Guidelines** → Prevent fraud and abuse.
 5. **Audit Preparedness** → Keep documentation to support all billed claims.
-

C. Why Medical Necessity + Compliance Matter in Billing

- Prevents **claim denials** → Insurance rejects claims if not medically necessary.
 - Protects against **fraud allegations** → Billing for unnecessary or unprovided services = fraud.
 - Ensures **timely reimbursement** → Clean, compliant claims are paid faster.
 - Safeguards **provider reputation** → Compliance protects against penalties and lawsuits.
-

☒ Summary:

- **Documentation (EHR/EMR)** → Foundation of billing; must be accurate and accessible.
- **Medical Necessity** → Only bill for services that are reasonable and required.
- **Compliance** → Following rules (HIPAA, OIG, coding standards) to avoid denials and legal issues.

1. Types of Health Insurance Plans

A. HMO (Health Maintenance Organization)

- Requires patients to choose a **Primary Care Physician (PCP)**.
- **Referrals** needed to see specialists.
- Lower cost, but limited provider network.
- Example: A patient with HMO must see their PCP before visiting a cardiologist.

B. PPO (Preferred Provider Organization)

- Flexible: Patients can see any provider, but cheaper **in-network**.
- No referral required for specialists.
- Higher premiums than HMO.
- Example: A PPO patient can go directly to a dermatologist without PCP approval.

C. Medicare

- Federal program (U.S.) for **65+ or disabled** patients.
- 4 Parts:
 - **Part A** → Hospital insurance.
 - **Part B** → Outpatient/doctor visits.
 - **Part C (Medicare Advantage)** → Private plans with extra benefits.
 - **Part D** → Prescription drug coverage.

D. Medicaid

- Joint federal + state program for **low-income individuals/families**.
- Eligibility varies by state.
- Covers hospital, doctor visits, long-term care, and sometimes dental/vision.

E. Workers' Compensation

- Insurance for employees injured **on the job**.
- Covers medical care + wage replacement.
- Example: A factory worker injured at work → bills go to Workers' Comp, not regular insurance.

2. Insurance Terminology

Term	Definition	Example
Premium	The monthly fee paid for insurance coverage.	\$400/month
Deductible	Amount the patient must pay out-of-pocket before insurance starts covering costs.	\$1,000/year deductible
Copay	Fixed amount patient pays for a service.	\$20 per doctor visit
Coinsurance	Patient pays a percentage of the cost after deductible is met.	Insurance pays 80%, patient pays 20%
Out-of-Pocket Maximum	The maximum a patient pays in a year; after that insurance covers 100%.	\$6,500/year

□ **Example:** If a patient has a \$1,000 deductible, they must pay that much before insurance starts paying. After that, they might pay **20% coinsurance** until they reach the **out-of-pocket max**.

3. Eligibility Verification & Insurance Coverage

Before submitting a claim, billers must **verify if the patient's insurance is active and what it covers**.

Steps for Eligibility Verification:

1. Collect patient's insurance card & demographics.
2. Contact payer (via portal, phone, or clearinghouse).
3. Verify:
 - Policy status (active/inactive).
 - Coverage type (hospital, outpatient, dental, vision).
 - Deductible, copays, coinsurance.
 - Pre-authorization requirements.
 - Network restrictions.
4. Document verification results in patient's record.

☐ **Why important?** Prevents claim denials due to inactive coverage or non-covered services.

4. Coordination of Benefits (COB)

- When a patient has **more than one insurance plan**, COB rules decide **which plan pays first**.

Order of Payment:

1. **Primary Insurance** → Pays first.
2. **Secondary Insurance** → Pays remaining eligible costs.
3. **Patient Responsibility** → Pays what insurance does not cover.

Examples:

- A child covered by both parents → Insurance of the parent whose birthday falls **earlier in the year** is primary (Birthday Rule).
- Medicare + Private Insurance → Usually private insurance pays first, Medicare is secondary.

☐ Billers must know COB rules to submit claims in correct order.

5. Prior Authorization & Referrals

A. Prior Authorization (Pre-Authorization/Pre-Certification)

- Some procedures, surgeries, or medications require **insurance approval before service is provided**.
- Example: MRI, expensive medications, elective surgeries.
- Billers often help providers submit requests.

- ☐ Without pre-authorization, claims are **denied**.

B. Referrals

- In **HMO plans**, patients need a referral from PCP to see a specialist.
- Example: A PCP must refer a patient to a cardiologist.
- Referral includes:
 - Patient info.
 - PCP info.
 - Reason for referral.

- ☐ **Billers must check if referrals were obtained** before submitting claims.
-

✓ Summary:

- Insurance plans differ (HMO = strict network, PPO = flexible, Medicare/Medicaid = government, Workers' Comp = job injury).
- Key terms (premium, deductible, copay, coinsurance) define **patient vs insurance responsibility**.
- Eligibility verification avoids **denials**.
- COB ensures claims go to the right payer.
- Prior authorization & referrals are **gatekeepers** for expensive care.

The Medical Billing Cycle

The **Medical Billing Cycle** is the process that ensures healthcare providers are paid for services. It begins **when a patient schedules an appointment** and ends **when the balance is paid** (by insurance + patient).

It usually has **7 key steps**:

1. Patient Registration & Data Entry

- **First step** in the billing cycle.
- Collect patient demographics & insurance details:
 - Full name, DOB, address, phone number.
 - Insurance provider, policy number, group number.
 - Employer information (if employer-based insurance).
- Enter details into **Practice Management (PM) system** or **EHR system**.
- Accuracy is critical → errors here = denied claims later.

- ☐ **Example Error:** If policy number is entered wrong → insurance automatically rejects the claim.
-

2. Insurance Verification Process

- Confirm if the patient's insurance is **active and valid**.
- Verify:
 - Coverage effective dates.
 - Deductibles, copays, coinsurance.
 - Network participation (in-network vs out-of-network).
 - Prior authorization/referral requirements.
- This can be done through:
 - Insurance portal (real-time check).
 - Clearinghouse.
 - Phone call to insurance provider.

☐ **Why important?** Saves time & prevents denials.

3. Charge Capture & Coding Basics

- After the patient's visit, provider documents all **services performed**.
- **Medical coder** assigns appropriate **ICD (diagnosis) and CPT/HCPCS (procedure) codes**.
- **Charge capture** = Converting services into **billable charges**.
- Billers review codes for compliance before claims submission.

☐ **Example:**

- ICD: J45.909 (asthma, unspecified).
 - CPT: 99213 (office visit, established patient).
-

4. Preparing and Submitting Claims (Paper vs. Electronic)

- Billers prepare claims with patient, provider, insurance, and coding info.
- Types of claims:
 - **Paper claim** → CMS-1500 (professional) or UB-04 (hospital).
 - **Electronic claim** → Sent via EDI (Electronic Data Interchange) through a clearinghouse.
- Clearinghouse checks for errors before forwarding to payer.

☐ **Today, 90%+ of claims are electronic** because they're faster and reduce errors.

5. Payment Posting & EOB (Explanation of Benefits)

- Once claim is processed, payer sends:
 - **EOB (Explanation of Benefits)** → Explains what was paid, denied, or adjusted.
 - **ERA (Electronic Remittance Advice)** → Digital version of EOB.
- Biller posts payments into patient account:
 - Insurance portion.
 - Patient responsibility (copay, deductible, coinsurance).

☐ **Example:**

- Total bill: \$200
 - Insurance paid: \$150
 - Patient owes: \$50 (deductible/copay).
-

6. Claim Follow-up and Denial Management

- Billers track claims until fully paid.
- If claim is denied:
 - Review denial reason (coding error, eligibility, missing info).
 - Correct and **resubmit (appeal)**.
- Denials can be:
 - **Soft denial** → fixable (e.g., missing modifier).
 - **Hard denial** → not billable (e.g., non-covered service).

☐ **Denial management = critical** to prevent revenue loss.

7. Collections & Patient Billing

- After insurance pays, remaining balance is billed to patient.
- Billers generate **patient statements** showing:
 - Charges, insurance payments, adjustments, patient responsibility.
- If unpaid:
 - Payment reminders → phone calls → final notices.
 - May be sent to **collections agency** (last resort).

☐ Good communication with patients avoids bad debt.

Flow of Medical Billing Cycle (Simplified)

1. Registration →
 2. Verification →
 3. Coding/Charge Capture →
 4. Claim Submission →
 5. Payment Posting →
 6. Denial Management →
 7. Collections
-

☒ **Summary for Learners:**

- **Front-end (Steps 1–3)** → Registration, verification, coding.
- **Mid-cycle (Steps 4–5)** → Claims submission, payment posting.
- **Back-end (Steps 6–7)** → Denial management, patient collections.

Claims Processing (Overview)

Claims processing = The workflow of **reviewing, validating, and adjudicating insurance claims**.

- After claims are submitted, insurance companies (payers) **review** them.
- They check eligibility, coding, coverage, and compliance.
- Final result = **Paid, Denied, or Rejected**.

☐ Claim types:

- **Clean Claim** → Submitted correctly, no errors → paid quickly.
 - **Rejected Claim** → Contained errors → never entered payer's system (fix & resubmit).
 - **Denied Claim** → Entered system but payment refused (appeal needed).
-

1. CMS-1500 Form Completion (Professional Billing)

- Used by **physicians, clinics, and individual providers**.
- Standard paper claim form for outpatient/professional services.
- Contains **33 fields (boxes)**.

Key Sections:

- **Patient & Insured Info (Boxes 1–13)** → Name, DOB, insurance details.
- **Provider Info (Boxes 14–33)** → NPI, Tax ID, address.
- **Diagnosis Codes (Box 21)** → ICD-10 codes.
- **Procedures & Charges (Box 24)** → CPT/HCPCS codes, modifiers, charges.
- **Signature & Date (Box 31)** → Provider certification.

☐ Billers must ensure **every field matches payer requirements** to avoid rejection.

2. UB-04 (Institutional Billing Basics)

- Also called **CMS-1450** form.
- Used by **hospitals, nursing facilities, inpatient/outpatient institutions**.
- Contains **81 fields**.

Key Sections:

- **Type of Bill (FL-4)** → Identifies claim type (hospital, skilled nursing, etc.).
- **Patient Info (FL 8–17)** → Name, DOB, address.
- **Occurrence & Value Codes (FL 31–41)** → Extra billing info (accidents, special cases).
- **Diagnosis Codes (FL 67–75)** → ICD-10.
- **Procedure Codes (FL 80–81)** → CPT/HCPCS.

□ **UB-04 is more complex** than CMS-1500 because it includes **room charges, facility fees, and institutional data**.

3. Clearinghouses & Electronic Claims Submission (EDI)

- **Clearinghouse** = A third-party system that checks and forwards electronic claims.
- Claims sent via **EDI (Electronic Data Interchange)** → HIPAA-compliant format.
- Functions:
 - Claim **scrubbing** (error-checking).
 - Transmitting claims securely to multiple payers.
 - Returning rejection/acceptance reports.

□ Without clearinghouses, providers would have to submit claims to **each payer separately**.

4. Claim Scrubbing & Error Prevention

- **Claim scrubbing** = Reviewing claims for errors before submission.
- Errors caught early = Higher chance of “clean claims.”

Common Errors Scrubbed:

- Missing/incorrect patient demographics.
- Wrong insurance ID number.
- Invalid ICD-10 or CPT codes.
- Missing modifiers.
- Duplicate claims.

□ Many Practice Management Systems (PMS) have **built-in scrubbing tools**.

5. Timely Filing Limits

- Every payer has a **deadline** for claim submission.
- If missed → claim automatically denied (no payment).

Typical Deadlines:

- Medicare → **12 months** from date of service.
- Medicaid → Varies by state (often 90–180 days).
- Private Insurance → Usually **90–180 days**.
- Workers' Compensation → Depends on state/employer rules.

☐ Billers must **track claim submission dates** carefully.

☒ Summary for Learners:

- **CMS-1500** = Physician/clinic claims.
- **UB-04** = Hospital/institutional claims.
- **Clearinghouses (EDI)** = Submit electronic claims, scrub errors.
- **Claim scrubbing** = Prevent denials before submission.
- **Timely filing** = Always submit within payer's deadline.

Reimbursement Methodologies (Overview)

Reimbursement = How healthcare providers are **paid for services** rendered.

- Insurance companies & government programs have different **payment models**.
 - Billers must know **how claims are reimbursed** to ensure accurate payment.
-

1. Fee-for-Service (FFS) vs. Value-Based Reimbursement

Fee-for-Service (Traditional Model)

- Providers paid **per service/procedure**.
- Example: Doctor performs 3 tests → paid separately for each test.
- **Pros:** Easy to understand, encourages more services.
- **Cons:** Can lead to unnecessary tests, **higher costs**.

Value-Based Reimbursement

- Providers paid based on **quality & outcomes** instead of volume.
- Example: Payment tied to reducing hospital readmissions or improving patient health.

- **Models include:** Pay-for-performance, bundled payments, accountable care.
 - **Pros:** Encourages efficiency & better patient care.
 - **Cons:** More complex reporting, risk for providers.
-

2. DRGs (Diagnosis Related Groups)

- Used for **inpatient hospital billing (Medicare & some private payers)**.
- Patients classified into **DRGs based on diagnosis, procedures, age, discharge status**.
- Each DRG has a **fixed reimbursement rate** regardless of actual hospital cost.

☐ Example:

- Appendectomy patient admitted → Falls into a DRG with set payment = \$5,000.
- If hospital cost = \$4,000 → hospital gains.
- If cost = \$6,000 → hospital loses.

☒ **Purpose:** Control costs, encourage efficiency.

3. APCs (Ambulatory Payment Classifications)

- Used for **outpatient hospital billing (Medicare Part B)**.
- Groups **similar outpatient services** together.
- Each APC has a **prospective fixed payment rate**.

☐ Example:

- MRI of brain → falls under a specific APC code with a fixed payment = \$800.
- Hospital receives that amount no matter actual cost.

☒ **Purpose:** Streamline outpatient billing & control spending.

4. Capitation & Global Billing

Capitation

- Provider is paid a **fixed amount per patient, per month/year** (whether patient seeks care or not).
- Example: Insurance pays doctor \$50 per member/month for 1,000 patients = \$50,000/month.
- Doctor must provide **all needed care within that payment**.

☒ Encourages preventive care, but risky if patients need expensive treatments.

Global Billing (Bundled Payments)

- One **lump sum payment** covers all services for an episode of care.

- Example: Global maternity billing → one payment covers prenatal visits, delivery, and postpartum care.

☑ Reduces fragmented billing & simplifies reimbursement.

5. Medicare & Medicaid Billing Guidelines

Medicare

- Federal insurance for **65+ and certain disabled individuals**.
- **Parts:**
 - Part A: Hospital insurance (inpatient, hospice, SNF).
 - Part B: Outpatient, physician services, DME.
 - Part C: Medicare Advantage (private plans).
 - Part D: Prescription drugs.
- Billing rules:
 - Use **CMS-1500 (professional)** or **UB-04 (institutional)**.
 - **Timely filing:** 12 months.
 - Must follow **Medicare Fee Schedule, DRGs, or APCs**.

Medicaid

- State & federal program for **low-income individuals/families**.
 - Coverage varies by **state rules**.
 - Typically requires **prior authorization** for many services.
 - Filing limits: often **90–180 days**.
 - Uses **CMS-1500/UB-04** like Medicare.
-

☑ Summary for Learners:

- **FFS = pay per service; Value-based = pay for quality/outcomes.**
- **DRGs = inpatient hospital fixed payments.**
- **APCs = outpatient hospital fixed payments.**
- **Capitation = fixed per patient, per month.**
- **Global Billing = lump sum for an episode of care.**
- **Medicare & Medicaid = strict billing rules, forms, and deadlines.**

Advanced Billing & Compliance

1. Fraud, Waste, and Abuse in Medical Billing

- **Fraud** = Intentional deception for financial gain.
 - Examples: Billing for services not provided, falsifying records, upcoding.
- **Waste** = Misuse of resources without intent.
 - Examples: Ordering unnecessary tests, inefficiency.
- **Abuse** = Practices that result in unnecessary costs.
 - Examples: Charging for non-covered services, misusing codes.

□ **Consequences:** Heavy fines, exclusion from Medicare/Medicaid, criminal charges.

2. OIG (Office of Inspector General) Compliance Guidelines

- **OIG Mission:** Protect Medicare & Medicaid from fraud, waste, and abuse.
- **Compliance Program Elements:**
 1. Written standards of conduct & policies.
 2. Compliance officer & committee.
 3. Training & education.
 4. Communication channels (hotlines).
 5. Internal monitoring & auditing.
 6. Enforcement & disciplinary actions.
 7. Corrective actions for identified issues.

□ **Billers must follow OIG guidance to stay compliant.**

3. Appeals and Grievance Process

- **Appeals:** When a claim is denied, providers can request **reconsideration**.
 - Levels:
 1. Reconsideration request (payer review).
 2. Formal written appeal.
 3. Independent review (for Medicare: ALJ, Council, Federal court).
 - **Grievance:** Patient complaint regarding service/coverage.
 - **Documentation is key** for successful appeals.
-

4. Corrective Action Plans (CAP) for Errors

- When errors or non-compliance are found, providers must create a **CAP**.
- **Steps in CAP:**
 1. Identify the error (audit results, denied claims).
 2. Develop action plan (training, policy changes).
 3. Implement corrective steps.
 4. Monitor results & report compliance.

□ Prevents repeated mistakes and shows regulators proactive compliance.

5. Audits and Documentation Requirements

- **Audits Types:**
 - Internal (self-audit by provider).
 - External (Medicare, Medicaid, or private payer audit).
 - RAC (Recovery Audit Contractor) for Medicare overpayments.
 - **Documentation Must Include:**
 - Patient demographics & insurance info.
 - Provider's notes & medical necessity.
 - Correct coding (ICD, CPT, HCPCS).
 - Proof of service delivery (signatures, test results).
 - **Best Practice:** "If it's not documented, it didn't happen."
-

✓ Summary for Learners:

- Fraud = intentional; Waste = careless; Abuse = improper practices.
- OIG guidelines = framework for compliance programs.
- Appeals process = multiple steps to overturn denials.
- Corrective Action Plans = structured fix for errors.
- Audits require **clear, complete documentation** to avoid penalties.

Technology in Medical Billing

1. Introduction to Practice Management Software (PMS)

- **Definition:** Software that helps healthcare providers manage daily operations.
 - **Functions for Billers:**
 - Patient scheduling & registration.
 - Insurance eligibility verification.
 - Charge entry & coding support.
 - Claims submission & tracking.
 - Payment posting & reporting.
 - **Examples:** Kareo, AdvancedMD, DrChrono.
-

2. Electronic Health Records (EHR) Billing Integration

- **EHR vs. EMR:**
 - **EMR (Electronic Medical Records):** Digital version of patient chart.
 - **EHR (Electronic Health Records):** Comprehensive patient info, shared across providers.
- **Billing Integration Benefits:**
 - Automatic charge capture from clinical notes.
 - Real-time coding suggestions.
 - Fewer billing errors → higher reimbursement.
 - Improves compliance with documentation requirements.

3. Common Billing Software

- **Kareo:** Cloud-based, ideal for small practices.
- **AdvancedMD:** Scalable, includes practice management + EHR.
- **eClinicalWorks:** Comprehensive EHR + billing + telehealth tools.
- **Athenahealth:** Known for revenue cycle management (RCM) services.
- **Epic & Cerner:** Used by large hospitals for integrated EHR + billing.

□ Learners should be introduced to **demo versions** or **training dashboards** to gain hands-on practice.

4. Telehealth Billing Guidelines

- **What is Telehealth?** Remote healthcare via video/phone.
 - **Billing Considerations:**
 - Use of **telehealth-specific CPT/HCPCS codes** (e.g., 99421–99423).
 - Modifier **95** → indicates synchronous telemedicine service.
 - Place of Service (POS) code **02 or 10** → telehealth visits.
 - **Challenges:**
 - Varying insurance coverage.
 - State-by-state policy differences.
 - Documentation of patient consent.
-

5. Emerging Trends in Medical Billing

- **Artificial Intelligence (AI) & Machine Learning:**
 - Predict claim denials before submission.
 - Automate coding & charge capture.
 - **Robotic Process Automation (RPA):**
 - Automates repetitive billing tasks (eligibility checks, claim status updates).
 - **Revenue Cycle Management (RCM) Platforms:**
 - End-to-end automation of billing → improves cash flow.
 - **Blockchain:**
 - Secure, tamper-proof patient data exchange.
 - **Analytics & Reporting Dashboards:**
 - Real-time KPIs (denial rates, days in A/R, collection rates).
-

✓ Summary for Learners:

- PMS and EHR systems **streamline billing** and reduce errors.
- Software like Kareo, AdvancedMD, and eClinicalWorks are **industry-standard tools**.
- Telehealth billing has **unique codes, modifiers, and rules**.
- Future of billing = **AI, automation, and advanced RCM solutions**.

Specialty Billing

1. Hospital Billing vs. Physician Billing

- **Hospital Billing (Institutional Billing):**
 - Uses **UB-04 (CMS-1450)** claim form.
 - Focus on **room charges, nursing, supplies, procedures**.
 - Often uses **DRGs (Diagnosis Related Groups)** for inpatient reimbursement.
 - More complex due to multiple departments/services.
 - **Physician Billing (Professional Billing):**
 - Uses **CMS-1500** claim form.
 - Focus on physician services, office visits, procedures.
 - Reimbursed via **CPT/HCPCS codes** and fee-for-service model.
-

2. Laboratory & Radiology Billing

- **Lab Billing:**
 - Uses CPT codes for tests (e.g., 80053 = comprehensive metabolic panel).
 - Frequent use of **medical necessity rules** → must match diagnosis codes (ICD).
 - Often billed on behalf of referring providers.
 - **Radiology Billing:**
 - Codes for both **professional component (reading the scan)** and **technical component (performing the scan)**.
 - Uses modifiers **-26** (professional) and **-TC** (technical).
 - Pre-authorization often required for advanced imaging (CT, MRI).
-

3. Mental Health & Behavioral Health Billing

- **Unique Codes:**
 - Psychiatric diagnostic evaluation → CPT 90791.
 - Individual psychotherapy → CPT 90832–90837.
 - Group therapy, family therapy, crisis intervention codes.
 - **Challenges:**
 - Session time-based billing.
 - Telehealth expanding in behavioral health.
 - Prior authorization often needed.
 - Limited coverage by some private insurers.
-

4. Dental Billing Basics

- Uses **CDT (Current Dental Terminology) Codes** instead of CPT/HCPCS.
- Common procedures: Exams, X-rays, fillings, crowns, extractions.
- Coverage differs → **medical insurance vs. dental insurance** (sometimes overlap, e.g., oral surgery billed to medical insurance).

- Dental claims use **ADA Dental Claim Form**, not CMS-1500.
-

5. Durable Medical Equipment (DME) Billing

- **What is DME?** Medical equipment prescribed for home use (wheelchairs, oxygen tanks, CPAP machines).
 - **Codes:** Billed using **HCPCS Level II codes** (e.g., E0100 = crutches).
 - **Requirements:**
 - Written physician order.
 - Proof of medical necessity.
 - Often requires **prior authorization**.
 - Rental vs. purchase billing differences.
 - **Payers:** Medicare, Medicaid, private insurance.
-

☑ Summary for Learners:

- **Hospital vs. physician billing:** UB-04 vs. CMS-1500, DRGs vs. CPT.
- **Lab & radiology billing** requires **medical necessity and modifiers**.
- **Mental health billing** is often **time-based** and growing with telehealth.
- **Dental billing** uses **CDT codes** and different claim forms.
- **DME billing** is highly regulated and often requires **authorizations + documentation**.

Career Preparation in Medical Billing

1. Job Roles in Medical Billing

- **Medical Biller**
 - Responsible for preparing and submitting claims.
 - Follows up on payments, denials, and patient balances.
 - **Accounts Receivable (AR) Specialist**
 - Focuses on tracking unpaid claims, denial management, and collections.
 - Works closely with insurance companies and patients.
 - **Revenue Cycle Management (RCM) Executive**
 - Manages the entire billing cycle → patient registration to collections.
 - Ensures compliance, reporting, and financial optimization for healthcare practices.
 - **Other Related Roles**
 - **Claims Processor** (insurance company side).
 - **Billing Analyst / Audit Specialist**.
 - **Medical Coding + Billing Specialist** (dual role in smaller organizations).
-

2. Resume & Interview Preparation

- **Resume Building Tips**
 - Highlight billing software skills (Kareo, AdvancedMD, eClinicalWorks, etc.).

- Emphasize knowledge of ICD, CPT, HCPCS, and claim forms (CMS-1500, UB-04).
 - Showcase accuracy, attention to detail, and compliance knowledge.
 - **Interview Preparation**
 - Common questions: “How do you handle claim denials?”, “What is your experience with EHRs?”, “How do you ensure HIPAA compliance?”
 - Role-play mock interviews.
 - STAR method for answering behavioral questions.
-

3. Freelance Medical Billing & Outsourcing Opportunities

- **Freelancing Platforms:** Upwork, Fiverr, LinkedIn, specialized medical billing outsourcing firms.
 - **Services Offered:**
 - End-to-end billing.
 - AR follow-up.
 - Denial management.
 - Patient billing services.
 - **Challenges & Opportunities**
 - Need for trust and data security.
 - Growing demand due to outsourcing trends in the US healthcare system.
-

4. Certification Exams

- **CMB (Certified Medical Biller)** – covers billing cycle, claim forms, compliance.
 - **CMRS (Certified Medical Reimbursement Specialist)** – offered by AMBA, focuses on advanced reimbursement and compliance.
 - **AAPC Billing Certification (CPB – Certified Professional Biller)** – most recognized credential; validates billing expertise and compliance skills.
 - **Preparation Tips**
 - Practice with mock exams.
 - Join study groups.
 - Review CMS guidelines and payer manuals.
-

5. Continuing Education & Professional Growth

- Importance of staying updated with **ICD, CPT, HCPCS annual updates**.
 - Attend **webinars, workshops, and conferences** on healthcare billing.
 - Join professional associations (AAPC, AMBA, AHIMA).
 - Pathway to management roles:
 - Medical Billing Supervisor → RCM Manager → Director of Revenue Cycle.
 - Transition opportunities:
 - Medical Coding, Healthcare Administration, Compliance Officer.
-

☑ **Summary for Learners:**

This module prepares learners for **real-world billing careers**, from entry-level positions to management. It guides them on **job hunting, freelancing, certifications, and lifelong growth**.